

D123

Family Nurse Practitioner Clinical Internship II

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

Method © 2026 Erik Daniel Lindquist — Course Mastery Coach, armored drill. Items drafted with AI assistance under his direction.

How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

A 46-year-old dental hygienist reports fatigue, weight gain, and feeling cold. She recently started a new exercise class and takes a daily fish-oil capsule. Labs show TSH 12.4 mIU/L (high) and free T4 0.5 ng/dL (low). What is the most likely diagnosis?

- A. Subclinical hypothyroidism.
- B. Hyperthyroidism, because TSH is high.
- C. No thyroid disease; symptoms are from the new exercise class.
- D. Overt primary hypothyroidism.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2.

A 34-year-old graphic designer with heavy menstrual periods reports fatigue. She is training for a half marathon and follows a vegetarian diet. Hemoglobin is 9.8 g/dL, MCV 71 fL, and ferritin 6 ng/mL. What is the most likely cause?

- A. Iron deficiency anemia.
- B. Vitamin B12 deficiency from her vegetarian diet.
- C. Anemia of chronic disease.
- D. Normal for endurance athletes; no workup needed.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3. [TRANSFER — reversed stem / different variable]

An older adult has a hemoglobin of 10.5 g/dL and an MCV of 112 fL. Which cause should be checked first?

- A. Thalassemia trait.
- B. Iron deficiency.
- C. Vitamin B12 or folate deficiency.
- D. Nothing; MCV rises normally with age.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4.

A 66-year-old retired pilot with type 2 diabetes takes metformin 2,000 mg daily. His A1C is 7.0% and he walks his dog twice a day. His eGFR has been 38 mL/min/1.73 m² on three checks over the past four months. What is the best metformin plan?

- A. Stop metformin immediately; it is unsafe with any CKD.
- B. Continue 2,000 mg daily, since his A1C is at goal.
- C. Continue metformin at a reduced dose (for example, no more than 1,000 mg daily) and monitor kidney function more often.
- D. Continue full dose because the eGFR puts him in stage G3a.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

5. [TRANSFER — reversed stem / different variable]

A patient's eGFR has been stable at 22 mL/min/1.73 m² for six months. What CKD stage is this, and what is the metformin status?

- A. Stage G5; metformin is contraindicated.
- B. Stage G3b; reduce the metformin dose.
- C. Stage G4; metformin can continue at full dose.
- D. Stage G4; metformin is contraindicated.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

6.

A 38-year-old warehouse supervisor has had low back pain for 10 days after moving furniture. He rates it 8 out of 10 and asks for an MRI because his brother-in-law had one. He has no leg weakness, numbness, fever, weight loss, cancer history, or bladder or bowel changes. What is the best plan?

- A. Prescribe strict bed rest for one week.
- B. Order an MRI because his pain is 8 out of 10.
- C. Order an MRI since his brother-in-law's MRI was helpful.
- D. No imaging now; advise staying active, heat, and an NSAID if appropriate, with return precautions.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

7.

A 29-year-old nursing student who recently moved into her own apartment completes a PHQ-9 with a total score of 12. She answers “several days” on item 9 about thoughts that she would be better off dead. She has two cats and plays in a weekend soccer league. What is the most important next step?

- A. Complete a suicide risk assessment during this visit.
- B. Recheck the PHQ-9 at her next routine visit.
- C. Start an antidepressant and follow up in four weeks, since the score is only moderate.
- D. Reassure her that pets and team sports protect against depression.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8. [TRANSFER — reversed stem / different variable]

A patient's PHQ-9 total is 17 and item 9 is scored zero. How is the depression severity classified?

- A. Moderately severe.
- B. Mild, because item 9 is zero.
- C. Moderate.
- D. Severe.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

A 61-year-old who restores classic cars has a 30 pack-year smoking history and chronic cough. His post-bronchodilator spirometry shows FEV1 2.1 L and FVC 3.5 L. His pulse oximetry is 96% and he still works full time. How should the result be interpreted?

- A. A restrictive pattern.
- B. Ratio 0.60, below 0.70, consistent with COPD.
- C. No COPD, because an FEV1 of 2.1 L is adequate.
- D. Ratio 1.67, normal.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10. [TRANSFER — reversed stem / different variable]

A patient's post-bronchodilator FVC is 4.0 L and her FEV1/FVC ratio is 0.65. What is her FEV1?

- A.** 2.8 L
- B.** About 6.2 L
- C.** 2.6 L
- D.** It cannot be calculated without a pre-bronchodilator test.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

A 27-year-old barista who has never been pregnant wants a birth control pill because her sister uses one. She has migraines with a visual aura about twice a month and does not smoke. Her blood pressure is 112/70. Which option is most appropriate?

- A.** The same combined pill her sister uses.
- B.** A progestin-only or non-hormonal method, such as a progestin pill, implant, or IUD.
- C.** A combined estrogen-progestin pill, since she is young and does not smoke.
- D.** A combined vaginal ring, because it is not a pill.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12.

A 70-year-old retired chef with shortness of breath on exertion has an echocardiogram showing an ejection fraction of 45%. He enjoys cooking for his grandchildren and has well-controlled blood pressure. How is his heart failure classified?

- A.** Heart failure with preserved ejection fraction.
- B.** Heart failure with mildly reduced ejection fraction.
- C.** Heart failure with reduced ejection fraction.
- D.** Reduced ejection fraction, because any EF below 50% is reduced.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer D Concept 1

A 46-year-old ~~dental hygienist~~ reports fatigue, weight gain, and feeling cold. ~~She recently started a new exercise class and takes a daily fish oil capsule.~~ Labs show TSH 12.4 mIU/L (high) and free T4 0.5 ng/dL (low). What is the most likely diagnosis?

Target word: thyroid diagnosis **Decides:** High TSH with low free T4.

A	Adjacent category	Subclinical means free T4 is normal; here it is low.
B	Inverted operation	In primary disease a high TSH means an underactive gland.
C	Camouflage	The abnormal labs, not the exercise detail, decide the diagnosis.
D	Correct	The pituitary raises TSH because the thyroid is making too little T4.

2. Answer A Concept 2

A 34-year-old ~~graphic designer~~ with heavy menstrual periods reports fatigue. ~~She is training for a half marathon~~ and follows a vegetarian diet. Hemoglobin is 9.8 g/dL, MCV 71 fL, and ferritin 6 ng/mL. What is the most likely cause?

Target word: type of anemia **Decides:** Low MCV with low ferritin.

A	Correct	Microcytosis with low ferritin confirms iron deficiency; heavy menses is a likely source.
B	Confusion set	B12 deficiency causes a high MCV, not a low one.
C	Adjacent category	Chronic disease anemia usually has normal or high ferritin.
D	Camouflage	Training does not explain a hemoglobin of 9.8 with ferritin of 6.

3. Answer C Concept 2 • transfer

An older adult has a hemoglobin of 10.5 g/dL and an MCV of 112 fL. Which cause should be checked first?

Target word: reverse: high MCV **Decides:** Macrocytic anemia.

A	Confusion set	Thalassemia trait is microcytic.
B	Inverted operation	Iron deficiency lowers MCV.
C	Correct	These are leading causes of macrocytic anemia.
D	Absolute language	An MCV of 112 with anemia is not normal aging.

4. Answer C Concept 3

A 66-year-old ~~retired pilot~~ with type 2 diabetes takes metformin 2,000 mg daily. His A1C is 7.0% and ~~he walks his dog twice a day.~~ His eGFR has been 38 mL/min/1.73 m² on three checks over the past four months. What is the best metformin plan?

Target word: metformin decision at stage G3b **Decides:** Sustained eGFR 38 = G3b; reduce dose, do not stop solely for this.

A	Absolute language	Metformin is not contraindicated until eGFR falls below 30.
B	Dropped element	Ignores the eGFR, which calls for a dose reduction.
C	Correct	At eGFR 30–44 metformin can continue at a lower dose; it is contraindicated only below 30.
D	Adjacent category	38 is stage G3b (30–44), not G3a (45–59).

5. Answer D Concept 3 • transfer

A patient's eGFR has been stable at 22 mL/min/1.73 m² for six months. What CKD stage is this, and what is the metformin status?

Target word: stage and drug status from the number **Decides:** 15–29 = G4; below 30 metformin is contraindicated.

A	Confusion set	G5 is below 15.
B	Adjacent category	G3b is 30–44.
C	Inverted operation	Right stage, wrong drug decision.
D	Correct	15–29 is G4, and metformin is not used below 30.

6. Answer D Concept 4

A 38-year-old warehouse supervisor has had low back pain for 10 days after moving furniture. He rates it 8 out of 10 and asks for an MRI ~~because his brother-in-law had one~~. He has no leg weakness, numbness, fever, weight loss, cancer history, or bladder or bowel changes. What is the best plan?

Target word: imaging decision **Decides:** Acute back pain without red flags.

A	Inverted operation	Staying active is recommended; bed rest slows recovery.
B	Adjacent category	Pain severity alone is not a red flag.
C	Camouflage	A relative's experience does not change the indication.
D	Correct	Imaging is not recommended in the first 6 weeks without red flags.

7. Answer A Concept 5

A 29-year-old nursing student ~~who recently moved into her own apartment~~ completes a PHQ-9 with a total score of 12. She answers “several days” on item 9 about thoughts that she would be better off dead. ~~She has two cats and plays in a weekend soccer league.~~ What is the most important next step?

Target word: priority action **Decides:** Any positive item 9 needs a same-visit suicide risk assessment.

A	Correct	Item 9 is positive; safety assessment comes before other planning.
B	Absolute language	Waiting treats a safety signal as routine.
C	Dropped element	Acts on the total score and skips the positive item 9.
D	Camouflage	Protective factors do not replace risk assessment.

8. Answer A Concept 5 • transfer

A patient's PHQ-9 total is 17 and item 9 is scored zero. How is the depression severity classified?

Target word: severity from total **Decides:** 15–19 = moderately severe.

A	Correct	17 falls in the 15–19 band.
B	Inverted operation	Item 9 flags safety; it does not lower the total.
C	Adjacent category	Moderate is 10–14.
D	Confusion set	Severe is 20–27.

9. Answer B Concept 6

A 61-year-old ~~who restores classic cars~~ has a 30 pack-year smoking history and chronic cough. His post-bronchodilator spirometry shows FEV1 2.1 L and FVC 3.5 L. ~~His pulse oximetry is 96% and he still works full time.~~ How should the result be interpreted?

Target word: interpret spirometry **Decides:** $FEV1/FVC = 2.1 / 3.5 = 0.60$.

A	Confusion set	A low FEV1/FVC ratio is obstructive; restriction shows a normal or high ratio.
B	Correct	$2.1 / 3.5 = 0.60$; post-bronchodilator ratio under 0.70 confirms obstruction.
C	Dropped element	Diagnosis rests on the ratio, not FEV1 alone.
D	Inverted operation	Divides FVC by FEV1.

10. Answer C Concept 6 · transfer

A patient's post-bronchodilator FVC is 4.0 L and her FEV1/FVC ratio is 0.65. What is her FEV1?

Target word: solve for FEV1 **Decides:** $FEV1 = \text{ratio} \times FVC$.

A	Confusion set	Uses the 0.70 diagnostic cutoff instead of her ratio.
B	Inverted operation	Divides FVC by the ratio instead of multiplying.
C	Correct	$0.65 \times 4.0 = 2.6$ L.
D	Absolute language	The ratio and FVC are enough.

11. Answer B Concept 7

A 27-year-old ~~barista~~ who has never been pregnant wants a birth control pill ~~because her sister uses one.~~ She has migraines with a visual aura about twice a month and does not smoke. ~~Her blood pressure is 112/70.~~ Which option is most appropriate?

Target word: method selection **Decides:** Migraine with aura contraindicates estrogen.

A	Camouflage	Her sister's method does not decide what is safe for her.
B	Correct	Estrogen raises stroke risk with migraine with aura.
C	Dropped element	Leaves out the migraine with aura, which alone rules out estrogen.
D	Confusion set	The ring also contains estrogen.

12. Answer B Concept 8

A 70-year-old ~~retired chef~~ with shortness of breath on exertion has an echocardiogram showing an ejection fraction of 45%. ~~He enjoys cooking for his grandchildren and has well-controlled blood pressure.~~ How is his heart failure classified?

Target word: HF category **Decides:** EF 41–49%.

A	Confusion set	HFpEF is 50% or more.
B	Correct	An EF of 41–49% is mildly reduced.
C	Adjacent category	HF rEF is 40% or less.
D	Absolute language	Current classification separates 41–49% as its own category.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	4	
Wrong stem number	A figure copied from the prompt that is not the answer.	0	
Adjacent category	The right idea in the wrong bucket.	7	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	7	
Absolute language	Always, never, guarantee — where judgment applies.	5	
Confusion set	Every option is real; the test is exact role-matching.	8	
Camouflage	Answers a stem fact that was never the question.	5	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Interpreting thyroid tests	In primary thyroid disease TSH moves opposite to free T4: high TSH with low free T4 = overt primary hypothyroidism; high TSH with normal free T4 = subclinical hypothyroidism.	Reading a high TSH as an overactive thyroid.	A TSH and free T4 pair with arrows in opposite directions.
2	Classifying anemia by MCV	Low MCV (microcytic) points first to iron deficiency (confirm with low ferritin); high MCV (macrocytic) points to B12 or folate deficiency, alcohol, or medications.	Giving iron for every anemia.	An MCV value with a hemoglobin below normal.
3	CKD staging and metformin	eGFR stages: G3a 45–59, G3b 30–44, G4 15–29, G5 under 15 (sustained over 3 months); metformin is not started at eGFR 30–45, is dose-reduced if already taken there, and is contraindicated below 30.	Stopping metformin at any drop in eGFR, or ignoring eGFR entirely.	A patient on metformin with an eGFR in the 30s or 40s.
4	Acute low back pain without red flags	For acute low back pain without red flags, avoid imaging in the first 6 weeks; advise staying active, heat, and NSAIDs if appropriate.	Ordering an MRI because the pain is severe or the patient asks.	Back pain under 6 weeks with no neuro deficit, cancer history, fever, or trauma.
5	PHQ-9 scoring and item 9	PHQ-9 totals: 5–9 mild, 10–14 moderate, 15–19 moderately severe, 20–27 severe; any positive answer on item 9 (thoughts of death or self-harm) calls for a same-visit suicide risk assessment regardless of total.	Letting a moderate total score override a positive item 9.	A PHQ-9 total plus a note about item 9.
6	Spirometry confirms COPD	A post-bronchodilator FEV1/FVC below 0.70 confirms persistent airflow obstruction consistent with COPD.	Diagnosing COPD from smoking history or symptoms alone, or reading FEV1 alone.	FEV1 and FVC values after a bronchodilator.
7	Estrogen contra indications in contraception	Combined estrogen-progestin methods are avoided with migraine with aura, age 35 or older and smoking, recent postpartum (under 21 days), or history of VTE; offer progestin-only or non-hormonal methods.	Prescribing a combined pill because the patient prefers a pill.	A contraception request plus migraine, smoking, age, or clot history.
8	Heart failure by ejection fraction	HF _r EF = EF 40% or less; HF with mildly reduced EF = 41–49%; HF _p EF = 50% or more; classification guides which therapies have the strongest evidence.	Calling any EF under 50% “reduced.”	An echocardiogram EF near the 40% or 50% cutoff.

Course scope (public description): Family Nurse Practitioner Clinical Internship II prepares the graduate nursing student to perform the role of an advanced practice nurse with the essential competencies and skills necessary to deliver primary care to individuals, families, and groups throughout the life span. In the precepted clinical setting, the student will combine competencies developed in preparatory advanced practice coursework to deliver consumer-centered healthcare. The student will conduct advanced health assessments and use the competencies of advanced pathophysiology, pharmacology, and health promotion for individuals and populations across the life span to build self-efficacy in individuals and groups, determine correct diagnoses, and establish plans of care that include consumer and population preferences. All MSN Core courses, Advanced Pathophysiology for the Advanced Practice Nurse, Advanced Pharmacology for the Advanced Practice Nurse, and Advanced Health Assessment for the Advanced Practice Nurse, FNP Specialty courses, and D122 Family Nurse Practitioner Internship I must be completed before taking this course.